Clinical Guideline: Cardiovascular Risk Assessment and Lipid Management

National Health Authority (Fictional) · Document NHA-CG-02 · Version 2.1 · Issued June 2026 · Synthetic Demonstration Edition

§1 Scope

This guideline covers cardiovascular risk assessment and lipid management for adults in the chronic-disease population, including patients with diabetes.

§2 Risk assessment

- All adults with diabetes should have a 10-year atherosclerotic cardiovascular disease (ASCVD) risk estimate recorded and refreshed at least every 12 months.
- Patients with established cardiovascular disease (coronary artery disease, heart failure, prior stroke or TIA, or peripheral arterial disease) are classified as Very High risk regardless of the calculated estimate.
- Calculated 10-year risk of 20% or above is classified High; 7.5% to below 20% is Moderate; below 7.5% is Low.

§3 LDL cholesterol targets

- Very High risk: LDL below 1.4 mmol/L.
- High risk: LDL below 1.8 mmol/L.
- Moderate risk: LDL below 2.6 mmol/L.
- Low risk: LDL below 3.0 mmol/L.

§4 Statin therapy

- High-intensity statin therapy is recommended for all High and Very High risk patients unless contraindicated or not tolerated.
- A High or Very High risk patient without an active statin prescription is recorded as an open statin care gap and prioritised for review within 8 weeks.
- If LDL remains above target on maximally tolerated statin, add ezetimibe; consider PCSK9 inhibition for Very High risk patients still above target.

§5 Antithrombotic therapy

- Antiplatelet therapy is recommended for secondary prevention in all patients with established atherosclerotic disease.
- Patients with atrial fibrillation should be assessed for stroke risk and anticoagulated unless contraindicated; an unassessed AF patient is recorded as an open anticoagulation care gap.

§6 Heart failure

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- Patients with heart failure with reduced ejection fraction should be established on the four pillars of therapy: a RAAS inhibitor (ACEi/ARB/ARNI), a beta-blocker, a mineralocorticoid receptor antagonist, and an SGLT2 inhibitor, titrated to target doses as tolerated.

§7 Human oversight of AI-assisted recommendations

AI-assisted risk stratification and therapy suggestions are advisory. Clinical sign-off is required before any change to therapy. See NHA-CG-01 §7.

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